STONEBRIDGE EAP SERVICES

Employee Assistance Programs · 410 Granite Way, Suite 300, Riverton · (555) 010-7700 · Fax (555) 010-7701

Confidential counseling record
EAP case SEAP-26-1187

June 10, 2026

Atty. Elijah Thorne
LSH Law Group
500 Commerce St, Suite 1200, Riverton

Re:	John Doe · DOB 08/14/1991 · Counseling Summary
Program:	Northgate Distribution Employee Assistance Program
Your request:	letter of 05/27/2026 · client's authorization signed 05/06/2026
Released:	treatment summary only; psychotherapy notes are not released

Dear Atty. Thorne:

At Mr. Doe's request and on his written authorization, I provide this summary of his counseling through his employer's Employee Assistance Program. The program is paid by his employer; there is no charge to Mr. Doe and no bill for these services.

Referral and sessions

Referred by Alana Reyes, MD, Lakeside Orthopedics, by fax on 04/20/2026. Six sessions by secure video, each 50 minutes: **04/24, 05/01, 05/08, 05/22, 05/29 and 06/05/2026**. The program's six-session benefit is now used.

Presenting concerns, in his words

"Every time I get in a car my chest goes tight." "On the 2nd we stopped at our corner, right where it happened, and a truck came up Birch and I couldn't breathe. I got out and walked." "I keep hearing the crunch when I try to sleep." "I didn't answer anybody. I didn't know how to say it without sounding stupid."

Assessment

Measure	Intake 04/24/2026	Session 6, 06/05/2026
GAD-7 (anxiety; 0–21)	15 (moderately severe)	7 (mild)
PHQ-9 (depression; 0–27)	12 (moderate)	5 (mild)
Sleep, self-reported	4–5 hours	6–7 hours

At intake Mr. Doe described panic symptoms as a car passenger, avoidance of car travel and of the intersection at Maple Hollow Rd and Birch Ave, intrusive memories and dreams of the collision, poor sleep, low mood and withdrawal from friends and recreation. He reported no anxiety, depression or counseling before the collision of 02/06/2026 and no thoughts of self-harm at any session. **Clinical impression:** symptoms consistent with an adjustment disorder with mixed anxiety and depressed mood (F43.23), which he relates, and his history supports, to the collision of 02/06/2026, with a marked worsening after the episode of 04/02/2026.

Treatment and progress

Cognitive-behavioral work: breathing and grounding skills, a graded plan for car travel (short daytime rides with his mother, then evening rides to therapy, then the intersection itself), and sleep routines. He attended every session. By 06/05/2026 he rode as a passenger to all his appointments, had driven past the corner with his mother twice, and was sleeping better. He still reports tension as a passenger at intersections and avoids the front of his house when he can.

Recommendations

Continue the skills practiced. If symptoms return or worsen, further counseling through his health plan is appropriate. I have not provided, and do not provide, any opinion beyond the counseling described here.

Sincerely,

Renee Castillo

Renee Castillo, MA, LPC
Licensed Professional Counselor, Stonebridge EAP Services